

BRIEF 5 — COMPLETE SOURCE MATRIX

F (Foundational). Six sources that frame the entire brief. They cover the paradigm's pre-history (F5, F6), its codification into a global model (F1, F2), and Nigeria's own CCT architecture from both the independent evaluation (F4a) and the current design document (F4b). F3 is the architect's own account of Progresa. These are the documents the brief relies on most heavily and returns to across multiple sections.

S (Country Stories). Eleven sources, one per country in the genealogy. Each tells the same five points: who started the programme, why, for what problem, how it was implemented, and what happened. Together they form the chronological spine of Sections 3 through 7. The countries are ordered by adoption date: Mexico (S1), Brazil (S2), Latin American regional diffusion (S3), Philippines (S4), Indonesia (S5), India (S6), Ghana (S7), Kenya (S8), Malawi (S9), South Africa (S10), Tanzania (S11).

N (Nigeria). Three sources specific to Nigeria's own experience. N1 is the first CCT attempt (COPE/SURE-P pilot, 2007-2015). N2 is investigative journalism documenting ground-truth delivery failures. N3 is a cross-reference to the sibling N-Power programme from Brief 4, included because N-Power's stipend erosion and ghost-name problems illustrate the same design weaknesses that NASSP-HUP faces.

X (Cross-cutting). Two sources that aggregate evidence across countries rather than within a single country. X1 is the systematic review on whether conditionality adds value over unconditional transfers. X2 is the Sub-Saharan Africa regional review showing how the CCT instrument mutates in weak-supply contexts. These serve Sections 9 and 10, where the brief pulls the country stories together and delivers the verdict.

Supporting papers. Five additional sources that strengthen specific claims: #135 on IO ghostwriting (Section 1 debate), #53 on the Burkina Faso conditionality RCT (Sections 7 and 10), #129 on whether Latin American CCTs over-expanded (Sections 5 and 10), #31 on long-term impacts (Section 9), and #150 on Progresa's birthweight effect (Section 3). These are not full 15-column tables; they are cited for the specific finding each contribute

F1. Fiszbein & Schady 2009 — Conditional Cash Transfers: Reducing Present and Future Poverty

Field	Extraction
ID	F1
Type	World Bank Policy Research Report (the definitive institutional statement)
Citation	Fiszbein, A. & Schady, N. (eds.) (2009). <i>Conditional Cash Transfers: Reducing Present and Future Poverty</i> . Washington, DC: World Bank.
Sections served	2, 3, 5, 9
Country	Global (with focus on Latin America)
Programme name	N/A (cross-country policy research report)
Year adopted → status	N/A
Form of CCT	Both (but emphasis on conditional)
Programme scale	CCTs operating in 27+ countries by 2008, up from 2 in 1997
What was achieved	(1) Codified the CCT paradigm: PMT targeting, conditionality on school + health, payment to mothers. (2) Documented positive impacts on consumption, school enrollment, health service utilisation across Latin America. (3) Established the "reduce present and future poverty simultaneously" rationale. (4) Acknowledged supply-side dependencies.
Where it fell short / limits	(1) Written from inside the paradigm — does not question whether IOs invented CCT. (2) Limited coverage of Africa and Asia. (3) Evidence on long-term outcomes still emerging at time of writing.
One-line what it adds	The institutional codification of CCT — the document that turned a Latin American experiment into a global policy template.

F2. Bastagli et al. 2016 — Cash Transfers: What Does the Evidence Say?

Field	Extraction
ID	F2
Type	Rigorous evidence review (ODI/OPM, 201 studies, 2000–2015)
Citation	Bastagli, F., Hagen-Zanker, J., Harman, L., Barca, V., Sturge, G., Schmidt, T. & Pellerano, L. (2016). <i>Cash transfers: what does the evidence say?</i> London: ODI.
Sections served	5, 6, 7, 9, 10
Country	Global (201 studies)

Programme name	N/A (synthesis across CCTs and UCTs)
Year adopted → status	N/A
Form of CCT	Both conditional and unconditional (compares)
Programme scale	130 LMICs have at least one UCT; 63 countries have at least one CCT (up from 2 in 1997 and 27 in 2008). 40/48 SSA countries now have a UCT, double the 2010 total.
What was achieved	Monetary poverty: 35 studies on total expenditure — 25 find increase; 31 on food expenditure — 23 find increase; FGT poverty measures — ~2/3 find significant decreases. Education: 20 studies on school attendance — 13 find significant increases. Learning outcomes: less clear, but 3 studies with significant findings all show improvements. Health & nutrition: CTs increase uptake of health services (9/15 significant); dietary diversity consistently increases (7/12 significant); anthropometric outcomes limited: stunting 5/13 significant, wasting 1/5, underweight 1/8. Savings, investment, production: Consistent positive effects on savings, livestock ownership, agricultural inputs. Employment: >half of studies find no significant impact on adult work; where significant, majority find increase; child labour consistently decreases. Empowerment: Transfers increase women's decision-making power and reduce physical abuse. Unintended effects: NO evidence of work disincentive or fertility increase.
Where it fell short / limits	(1) Third-order (medium/long-term) outcomes have limited evidence. (2) Conditionality: hard to disentangle which aspect drives results; messaging matters as much as enforcement. (3) Complementary interventions needed for anthropometric improvements. (4) Payment mechanisms and grievance systems — evidence base small. (5) Sex-disaggregated evidence sparse. (6) Time use trade-offs for women under-studied.
One-line what it adds	The most rigorous cross-country evidence base on what cash transfers actually do across 6 outcome domains — the backbone of Section 9. GAP FLAG: F2 PDF was extracted successfully but the original URL was broken; file is in workspace at.

F3. Levy & Rodríguez 2006 — Progress Against Poverty / Sin Herencia de Pobreza

Field	Extraction
ID	F3
Type	Book (IADB/Brookings Institution Press)

Citation	Levy, S. and Rodríguez, E. (2005/2006). <i>Sin herencia de pobreza: El programa Progresa-Oportunidades de México</i> . Mexico: IADB/Editorial Planeta. English edition: <i>Progress Against Poverty: Sustaining Mexico's Progresa-Oportunidades Program</i> . Washington, DC: Brookings Institution Press, 2006.
Note	Spanish PDF (249 pages) obtained via Wayback Machine from IADB publications. English edition paywalled; Spanish edition translated and extracted.
Sections served	1, 3, 9
Country	Mexico
Programme name	Progresa / Oportunidades / Prospera
Year adopted → status	1997 launched → 2019 replaced
Form of CCT	Classical conditional (education + health + nutrition co-responsibility)
Book structure	13 chapters: (1) Strategy against extreme poverty, (2) Profile of extreme poverty, (3) Pre-1995 programmes and reasons for change, (4) New orientation for human capital, (5) Concentrating efforts on the neediest, (6) Pre-Progresa work, (7) Objectives/components/scope, (8) Parallel/complementary programmes, (9) Transition from old programmes, (10) Income distribution, (11) Impact evaluation, (12) Transition under Fox, (13) Final considerations.
Key arguments (translated from Spanish)	From Introduction: In early 1995, despite decades of effort, poverty in Mexico was grave: over one-fifth of families lacked income for the basic food basket; 10 million Mexicans lacked regular health services; 1.5 million children aged 6-14 did not attend school. The 1994-95 peso crisis could worsen this. Four challenges: (1) one in five Mexicans in extreme poverty, 40% of them children; (2) the relative ineffectiveness of existing federal instruments, especially food subsidies; (3) the crisis would limit fiscal space for social spending; (4) the probability that extreme poverty would deepen. Economic growth alone was insufficient. In 1997, Progresa was launched as part of a broader strategy.
	From Chapter 4 (Design Rationale): The deficiencies in education, nutrition and health are simultaneously cause and effect of marginalisation, and there is strong interdependence between them. Transferring more income to poor families is not sufficient to improve nutrition if health conditions (diarrhoea, intestinal disease) prevent nutrient absorption. More food does not directly translate to better nutrition. Information problems affect how families allocate additional income. Within-household distribution of food is unequal. For the most vulnerable members (under-5s, pregnant/lactating

	women), greater intra-household targeting is needed. This is the argument for conditionality and co-responsibility: cash alone does not guarantee human capital investment; complementary requirements ensure that additional income translates into health visits, school attendance, and nutrition monitoring.
	From Chapter 6 (Pre-Progreso Work): In March 1995, the economic crisis opened a window of opportunity to reform anti-poverty instruments. In October 1995, a pilot programme began in Campeche, testing two innovations: (i) replacing in-kind transfers (milk, tortilla) with cash transfers via electronic card usable at affiliated shops; (ii) conditioning the transfer on regular health clinic attendance by vulnerable members (pregnant women, under-5s). The pilot covered 31,287 families. Evaluation showed 97% of beneficiaries preferred the new programme over the two it replaced. Cash was preferred to in-kind; health services were valued; attendance increased. However, the pilot was incomplete: no link to education, transfer was no larger than before, operational coordination was complex. From this, Levy concluded the programme should not be generalised as-is, but should lay the groundwork for a more ambitious effort. The Campeche pilot is the direct precursor to Progreso.
	From Chapter 5 (Targeting): The book separates two conceptually distinct questions: (1) why give demand subsidies tied to beneficiary behaviour (conditionality), and (2) why concentrate these efforts only on a subset of the population (targeting). These are often confused but should be analysed separately.
	From Chapter 13 (Conclusions): Oportunidades shows that the problem of poor families is not "access to food" but lack of income; that poor families spend additional income responsibly; and that food distribution or price subsidies are not the most adequate way to reduce malnutrition. The programme bridges the dilemma between welfare assistance and productive support, between present consumption and future consumption. It is also an instrument for reducing income inequality and protecting the poor against shocks. Oportunidades cannot solve all extreme poverty problems: it addresses human capital and income, but other instruments are needed for balanced regional development, credit access, housing. By 2004, the programme covered 5 million families (one in four Mexicans), with a budget of approximately USD 2.531 billion, the largest anti-poverty budget in Mexican history. This suggests the gains of the first eight years are significant, and that one should begin thinking about constructing a permanent social security system for all. What must remain unaltered is the commitment to eliminate extreme poverty.

What was achieved	(1) The architect's own account of Progresas design rationale, co-responsibility logic, and the Campeche pilot that preceded it. (2) Detailed argument for why cash alone is insufficient without conditionality: health mediates the food-nutrition link; information problems distort spending; intra-household distribution is unequal. (3) Campeche pilot (1995) as the direct precursor: cash replaced in-kind, conditionality on health attendance, 97% beneficiary preference. (4) The targeting-conditionality distinction clearly separated. (5) By 2004: 5M families, one in four Mexicans, USD 2.5B budget.
Where it fell short / limits	(1) Book concludes in 2003; does not cover Fox administration expansion, Calderon changes, or AMLO replacement. (2) Written from inside the programme; does not address the IO ghostwriting critique (Heimo & Syväterä 2022). (3) Learning outcomes acknowledged as limited but not deeply analysed. (4) Does not address the political sustainability question that later proved relevant when AMLO replaced CCT with unconditional pension.
One-line what it adds	The architect's own voice: why conditionality, why cash not in-kind, why co-responsibility, why targeting, and the Campeche pilot that proved it could work before Progresas was named. Essential for Section 3 and the Section 1 debate (Levy as counterweight to Heimo & Syväterä).

F4a. NASSP ICR Review 2024

Field	Extraction
ID	F4a
Type	World Bank IEG Independent Evaluation (Implementation Completion and Results Report Review)
Citation	World Bank IEG (2024). "Nigeria — National Social Safety Net Project (P151488): ICR Review."
Borrower	Federal Republic of Nigeria
Implementing Agency	FMHADMSD (Federal Ministry of Humanitarian Affairs, Disaster Management and Social Development)
Original IDA Credit	USD 500 million + GoN counterpart (USD 322.5M from Abacha Restituted Funds)
Project ID	P151488
Sections served	8, 9
Country	Nigeria

Programme name	NASSP-HUP (National Social Safety Net Project — Household Upraising Programme)
Year adopted → status	2016 approved → 2022 closed (replaced by NASSP Scale-Up P176935)
Form of CCT	Soft conditional (top-up co-responsibility transfers for school + health, but only 6% of HH received top-ups vs 80% target)
Programme scale	9,666,420 direct beneficiaries (target 5,000,000 — greatly exceeded). NSR: 15,673,296 households. 37 states (target 24 — exceeded).
ICR Outcome Rating	SATISFACTORY
Overall Efficacy	SUBSTANTIAL
Efficiency Rating	SUBSTANTIAL
M&E Quality Rating	SUBSTANTIAL
Bank Performance	SATISFACTORY
Risk to Development Outcome	Moderate
Targeting accuracy	Only 53% of beneficiaries in bottom 2 quintiles (target 80% — FAILED). Significant inclusion error.
Targeting fairness	89.1% reported fair (target 90% — slightly short)
Female transfer recipients	94% (target 85% — exceeded)
Top-up (co-responsibility) transfers	Only 6% of HH received (target 80% — FAILED). Supply-side constraints in education/health.
Electronic payments	100% (target 85% — exceeded)
Payment timeliness	90% within 1 month (target met)
Impact evaluation	Preliminary results show positive impacts on household savings, food security, farmland, livestock, women's happiness/autonomy. Final impact evaluation report NOT available at ICR drafting.
M&E weakness	Original baseline was actually a midterm survey (delivered 2019, after cash transfers already started). Impact evaluation not finalized at project close.
What was achieved	(1) Coverage vastly exceeded targets (9.7M vs 5M). (2) 37 states covered vs 24 target. (3) 94% female recipients. (4) 100% electronic payments. (5) Preliminary positive impacts on savings, food security, assets, women's autonomy.

Where it fell short / limits	(1) Targeting accuracy only 53% in bottom 2 quintiles — major inclusion error. (2) Top-up co-responsibility transfers failed (6% vs 80% target) — conditionality collapsed in practice. (3) Impact evaluation incomplete at close. (4) Baseline was actually midterm — undermines evaluation rigour.
One-line what it adds	The independent verdict on NASSP-HUP: scale exceeded, targeting failed, conditionality collapsed, impact evidence still missing.

F4b. NASSP Scale-Up PAD 2024

Field	Extraction
ID	F4b
Type	World Bank Project Appraisal Document (PAD)
Citation	World Bank (2024). "National Social Safety Net Program — Scale Up (P176935): Project Appraisal Document." PAD4674.
Borrower	Federal Republic of Nigeria
Implementing Agency	FMHADMSD
Total Project Cost	USD 800 million (IDA Credit)
Project ID	P176935
Sections served	8, 11
Country	Nigeria
Programme name	NASSP Scale-Up (Economic Shock Responsive + Extended Regular CT)
Year adopted → status	2022 approved → Active (under Renewed Hope framing)
Form of CCT	Unconditional (shock-responsive and extended regular transfers have no enforced conditions)
Components	Component 1: Economic Shock Responsive Cash Transfer (ESR-CT) — USD 600M. Time-limited CTs of NGN 5,000/month (~USD 12.17) per household for 6 months. Subcomponent 1.1: Rural ESR-CT via NSR (USD 235M, targeting 3.2M rural households). Subcomponent 1.2: Urban ESR-CT via RRR expansion (USD 365M, targeting 5M urban households). Component 2: Extended Regular Cash Transfer (ER-CT) — USD 147M. Continues NASSP regular-CT for existing 1.8M beneficiaries for at least 2 years total, plus exit grant. Transfer: NGN 5,000/month. Component 3: Delivery System Strengthening + Project Management — USD 53M. RRR expansion, payment digitisation, MIS integration, targeting improvement (machine learning/satellite imagery), impact evaluation, GRM, cross-cutting issues.

Transfer amount	NGN 5,000/month (~USD 12.17) per household — constitutes 6–7% of average monthly consumption of poor households
NSR coverage	10.1 million households (42.7M individuals) by September 2021
RRR coverage	1.75 million successful urban registrations; 1,163 poorest urban wards targeted
Target beneficiaries	Up to 8.2M households (3.2M rural + 5M urban) under ESR-CT; 1.8M existing under ER-CT
Risk ratings (SORT)	Overall: Substantial . Political & Governance: Substantial . Macroeconomic: Substantial . Sector Strategies: Moderate. Technical Design: Substantial . Institutional Capacity: Moderate. Fiduciary: Substantial . Environment & Social: Substantial . Stakeholders: Moderate.
PDO	"Expand coverage of shock responsive safety net support among the poor and vulnerable and strengthen the national safety net delivery system."
Key innovations	(1) RRR uses satellite imagery + call detail records + USSD mobile registration for urban targeting. (2) Output-Based disbursement to states. (3) Independent verification of beneficiary counts. (4) Digital payment via NUBAN accounts/PSPs.
What was achieved	(1) USD 800M operation — largest IDA social protection credit in Nigeria. (2) First large-scale shock-responsive safety net. (3) Digital-first delivery architecture. (4) Builds on NASSP infrastructure (NSR, RRR, NASSCO, NCTO).
Where it fell short / limits	(1) Transfer of NGN 5,000/month is only 6–7% of poor household consumption — well below 20% threshold. (2) Five of nine risk categories rated Substantial. (3) Conditionality entirely absent — this is a UCT, not CCT. (4) Payment delivery still dependent on PSP contracts and CBN cooperation. (5) Impact evaluation planned but not guaranteed.
One-line what it adds	The current architecture: USD 800M, unconditional, shock-responsive, digital-first — but with Substantial risk ratings and transfers far below the impact threshold.

F5. Fenwick 2009 — Avoiding Governors

Field	Extraction
ID	F5
Type	Peer-reviewed article (Cambridge University Press)
Citation	Fenwick, M. (2009). "Avoiding Governors: The Success of Bolsa Família." <i>Latin American Research Review</i> 44(2). Also earlier version in <i>University of Cambridge ESRC Conference</i> .
Sections served	1, 4

Country	Brazil
Programme name	Bolsa Escola (Brasilia 1995, Campinas 1995) → Bolsa Família (2003)
What was achieved	(1) Brasília 1995: Governor Cristovam Buarque created Bolsa Escola — a minimum-income-for-school-attendance programme at municipal level, BEFORE the World Bank was involved. (2) Campinas 1995: Mayor José Roberto Magalhães Teixeira independently created a similar programme. (3) The "avoiding governors" mechanism: Municipal-level innovation bypassed state-level political opposition. Federal government later adopted and scaled. (4) Proof that CCT was a Southern local invention , not a Washington creation.
Where it fell short / limits	(1) Municipal experiments were small-scale. (2) The article focuses on political mechanism, not impact evaluation.
One-line what it adds	The pre-history proof: Southern local political leaders invented CCT before the World Bank started writing cheques. Essential for Section 1 argument.

F6. Hanlon, Barrientos & Hulme 2010 — Just Give Money to the Poor

Field	Extraction
ID	F6
Type	Book (Kumarian Press / excerpt)
Citation	Hanlon, J., Barrientos, A. & Hulme, D. (2010). <i>Just Give Money to the Poor: The Development Revolution from the Global South</i> . Sterling, VA: Kumarian Press.
Sections served	1, 2, 7
Country	Global South (comparative)
What was achieved	(1) Four conclusions: (a) Cash transfers reduce poverty and inequality. (b) They are affordable. (c) They are administratively feasible even in low-capacity states. (d) They are politically sustainable. (2) Paradigm shift argument: The Global South led a "development revolution" that reversed the "poor are lazy" and "poor cannot manage money" assumptions. (3) Scale data: CT programmes expanding rapidly across SSA, Asia, Latin America. (4) Poverty reduction evidence: Mozambique, South Africa, Brazil, Mexico all show measurable poverty reduction from CTs.
Where it fell short / limits	(1) Tends toward advocacy — less rigorous than Bastagli ODI review. (2) UCT-leaning perspective — less balanced on conditionality debate.
One-line what it adds	The paradigm-level argument that the Global South led the cash transfer revolution — the foundational text for Section 1.

S1. Mexico — Progres/Oportunidades

Field	Extraction
ID	S1
Type	NBER Working Paper (long-term impact evaluation)
Citation	Parker, S.W. & Vogl, T. (2018/2021). "Do Conditional Cash Transfers Improve Economic Outcomes in the Next Generation? Evidence from Mexico." NBER Working Paper 24303.
Sections served	3, 9
Country	Mexico
Programme name	Progres/Oportunidades / Prospera
Year adopted → status	1997 launched → 2019 replaced by unconditional pension under AMLO
Form of CCT	Classical conditional (school attendance + health visits + nutrition)
Introduced by / rationale	Santiago Levy (Deputy Finance Minister) + President Ernesto Zedillo after 1994 peso crisis. Rationale: replace inefficient food subsidies with targeted cash, reduce present and future poverty simultaneously. Randomised rollout (1998) seeded the global evidence base.
Programme scale	6.5 million families at peak (~25% of Mexican population). Coverage in 3 poorest states: >50% of families.
Transfer size in USD	~USD 40–90/household/month (varying by composition and grade level)
What was achieved	(1) Randomised evaluation became the template for global CCT evidence. (2) Short-term: Consumption +7–10%, school enrollment +12% secondary, stunting reduced, child labour decreased. (3) Long-term (Parker & Vogl 2018): Childhood exposure improves women's outcomes in early adulthood — increases in educational attainment, geographic mobility, labor market performance, and household living standards. For men, effects are smaller. (4) Birthweight impact (Barber & Gertler 2008, #150): +127.3g birthweight, 4.6pp reduction in low birthweight.
Where it fell short / limits	(1) Learning outcomes stagnant — enrollment ↑ but test scores modest. (2) Urban rollout weaker than rural. (3) No lasting employment gains for men. (4) 2019 replacement under AMLO with unconditional pension suggests political fragility of conditionality. (5) Supply-side dependency — where schools/clinics poor, conditionality cannot produce intended gains.
One-line what it adds	The origin story with 20+ years of follow-up data — the paradigm's founding experiment and its long-term returns.

S2. Brazil — Bolsa Família

Field	Extraction
ID	S2
Type	World Bank SP Discussion Paper (design and implementation)
Citation	Lindert, K., Linder, A., Hobbs, J. & de la Brière, B. (2007). "The Nuts and Bolts of Brazil's Bolsa Família Program." World Bank SP Discussion Paper 0709.
Sections served	4, 9
Country	Brazil
Programme name	Bolsa Família (from Bolsa Escola 1995)
Year adopted → status	2003 consolidated → Active (restructured 2023 under Lula)
Form of CCT	Soft conditional (school attendance + health visits; enforcement varies by municipality)
Introduced by / rationale	Lula (PT, 2003–2006) unified 4 existing programmes. Rationale: poverty reduction manifesto commitment, unified register (Cadastro Único), minimum income floor for extreme poverty.
Programme scale	Largest CCT in the world. 13.9M families (~54M people, ~25% of Brazilians) by 2020.
Transfer size in USD	Two components: Basic benefit BRL 89/month (~USD 20) for extremely poor; Variable benefit BRL 41/month per child (~USD 9–12). Total typically USD 20–45/household/month.
What was achieved	(1) Decentralised implementation works —!across 5,564 municipalities. (2) Self-declaration plus municipal validation for targeting. (3) Soft conditionality enforced variably. (4) Inequality reduction: CCTs responsible for ~21% of fall in Brazilian Gini index mid-1990s to mid-2000s (Soares et al. 2007, #46). (5) 20+ years continuous operation across 4 administrations.
Where it fell short / limits	(1) Conditionality enforcement weak in practice. (2) Learning outcomes limited. (3) Transfer size small relative to poverty gap.
One-line what it adds	The scale-up story: how the world's largest CCT operates in practice, with decentralised implementation enabling federation-wide coverage.

S3. Latin American Regional Diffusion

Field	Extraction
ID	S3
Type	Regional overview (CEPAL Review)
Citation	Villatoro, P. (2005). "Conditional Cash Transfer Programmes: Experiences from Latin America." CEPAL Review 86: 83–103.

Sections served	5
Country	Latin America (regional)
Programme name	Multiple: Colombia Familias en Acción, Nicaragua RPS, Honduras PRAF, Jamaica PATH, Ecuador BDH, Chile Solidario, Peru Juntos, Bolivia Bono Juancito Pinto, Uruguay AF
Year adopted → status	1990–2008 various
Form of CCT	Variable4 (mostly classical conditional, some hybrid)
Programme scale	Combined ~15–20M households across region by 2010
Transfer size in USD	Variable USD 15–70/month by country
What was achieved	(1) Rapid diffusion: From 2 countries (1997) to most of Latin America by 2005. (2) Common design pattern: PMT targeting, conditionality on school + health, payment to mothers. (3) Nicaragua RPS (#13) : Achieved goals but short-lived — closed when domestic support evaporated. (4) Honduras PRAF (#14) : CCT increased incumbent mayor re-election by 39% — political capture risk. (5) Bolivia Bono (#58) : Universalist alternative — impacts weak due to poor supply-side.
Where it fell short / limits	(1) Variable outcomes — same playbook, different results depending on institutional capacity. (2) Nicaragua shows CCT can die if domestic constituencies don't build. (3) Honduras shows CCT can be captured electorally. (4) Bolivia shows CCT fails where supply-side services absent. (5) "Did CCTs go too far?" (#129) — question of over-expansion.
One-line what it adds	The diffusion pattern from origin to region-wide adoption in 10 years — and the variable outcomes the same template produces in different institutional contexts.

S4. Philippines — Pantawid Pamilyang Pilipino Program (4Ps)

Field	Extraction
ID	S4
Type	Programme overview (World Bank)
Citation	World Bank (2014). "Overview of the Philippines' Conditional Cash Transfer Program: The Pantawid Pamilyang Pilipino Program."
Sections served	6, 9
Country	Philippines
Programme name	Pantawid Pamilyang Pilipino Program (4Ps)

Year adopted → status	2007 → Active
Form of CCT	Classical conditional (school attendance + health visits + nutrition)
Introduced by / rationale	Arroyo administration + World Bank credit. Rationale: MDG poverty commitments.
Programme scale	~4.4 million households (2020)
Transfer size in USD	USD 30–45/household/month
What was achieved	(1) Largest Asian CCT. (2) School enrollment and attendance increased. (3) Health service utilisation improved. (4) Tests whether Latin American design travels to Asian institutional context.
Where it fell short / limits	(1) Limits to transformational potential — entitlement and agency constrained. (2) Conditionality enforcement challenges in remote areas. (3) Supply-side gaps. (4) Political vulnerability to patronage.
One-line what it adds	Largest Asian CCT — tests whether Latin American design travels to Asia.

S5. Indonesia — PKH

Field	Extraction
ID	S5
Type	Clustered RCT evaluation
Citation	Kusuma, D., Thabrany, H., Hidayat, B., McConnell, M., Berman, P. & Cohen, J. (2017). "New Evidence on the Impact of Large-scale Conditional Cash Transfers on Child Vaccination Rates." <i>Journal of Development Economics</i> .
Sections served	6, 9
Country	Indonesia
Programme name	PKH (Program Keluarga Harapan)
Year adopted → status	2007 → Active
Form of CCT	Classical conditional (maternal health + child education + immunisation)
Introduced by / rationale	Yudhoyono administration + World Bank technical support. Rationale: post-fuel-subsidy-reform social protection.
Programme scale	10 million families (2022)
Transfer size in USD	USD 30–50/household/month (average)

What was achieved	(1) Clustered RCT evidence: PKH increased complete vaccination rates among children under 2. (2) Second-largest Asian CCT. (3) Strong evaluation evidence base.
Where it fell short / limits	(1) Supply-side constraints limit conditionality effectiveness. (2) Scale-up strains administrative capacity. (3) Comparative evidence with UCT programmes (Jamkesmas) limited.
One-line what it adds	Second-largest Asian CCT with strong RCT evaluation — evidence that CCT can work for health outcomes beyond education; education.

S6. India — Janani Suraksha Yojana (JSY)

Field	Extraction
ID	S6
Type	Replication study (peer-reviewed)
Citation	Carvalho, N. & Rokicki, S. (2019). "The Impact of India's Janani Suraksha Yojana Conditional Cash Transfer Programme: A Replication Study." <i>Journal of Development Studies</i> 55(5): 989–1006.
Sections served	6, 9
Country	India
Programme name	Janani Suraksha Yojana (JSY)
Year adopted → status	2005 → Active
Form of CCT	Classical conditional (single condition: institutional delivery)
Introduced by / rationale	Government of India, National Rural Health Mission. Rationale: reduce maternal mortality by incentivising institutional delivery. Budget: US\$342 million (2009–10).
Programme scale	~10+ million beneficiaries/year (grew from 739,000 in 2005–06 to 11M+ in 2010–11)
Transfer size in USD	USD 8–17 one-time per institutional delivery (varies by state)
What was achieved	(1) Lim et al. (2010) Lancet results are replicable and robust. (2) In-facility delivery and skilled birth attendance increased. (3) Meaningful heterogeneity across states. (4) Neonatal mortality reduction in some states.
Where it fell short / limits	(1) Slow and uneven implementation. (2) Corruption documented. (3) Poor quality of care in facilities — institutional delivery ↑ but quality not guaranteed. (4) Out-of-pocket costs remain despite the transfer. (5) Single-condition CCT is narrow.
One-line what it adds	Single-outcome CCT showing what narrow conditionality can and cannot achieve — the Asian variant of the instrument.

S7. Ghana — LEAP

Field	Extraction
ID	S7
Type	UNICEF Transfer Project evaluation (mixed methods, regression discontinuity design)
Citation	Handa, S., de Hoop, J., Palermo, T., et al. (2016). "Ghana LEAP 1000 Programme: Baseline Evaluation Report." UNICEF Office of Research — Innocenti. Plus: de Groot, R. (2016). "Ghana LEAP 1000 Impact Evaluation: Analysis of Transfer Size and Estimated Impacts." Research Brief 11, The Transfer Project / UNICEF Innocenti.
Sections served	7, 9
Country	Ghana
Programme name	LEAP (Livelihood Empowerment Against Poverty)
Year adopted → status	2008 → Active
Form of CCT	Soft conditional (school + health for children, unconditional for 3 elderly and disabled). LEAP 1000 extension targets pregnant women and mothers with children under 15 months.
Introduced by / rationale	Government of Ghana (Department of Social Welfare, MoGCSP) + DFID + World Bank. Rationale: Africa's flagship poverty alleviation programme. LEAP 1000 added in 2015 targeting first 1000 days.
Programme scale	Mainstream LEAP: 116,000+ households (October 2015). Target was 164,000 over 5 years. LEAP 1000 evaluation sample: 1,262 HH below PMT cut-off + 1,235 above.
Transfer size in USD	GH¢8–15/beneficiary/month (2010–2016). By 2016, eroded to US\$5–9/month. Transfer constitutes 16% of baseline consumption on average (median 13%). Below the ~20% threshold for widespread impacts.
What was achieved	(1) Operational since 2008 — Africa's longest-running national CT. (2) LEAP 1000 uses rigorous RDD evaluation. (3) Endline findings (2020): Raised consumption, decreased poverty, no fertility increase, increased ANC visits, reduced intimate partner violence, increased NHISG enrollment, no significant impact on stunting. (4) Transfer regularity matters: each additional payment improves food (+30%), healthcare access (+12%), school participation (+22%), housing (+17%), social inclusion (+48%). (5) Transfer too small for widespread impacts at 13% median share of consumption.
Where it fell short / limits	(1) Transfer size too low compared to regional peers. (2) No automatic indexation. (3) Coverage below target (116K vs 164K). (4) Soft conditionality — enforcement weak. (5) No stunting impact (supply-side constraint).

One-line what it adds	Africa's flagship, tests whether the instrument works with weakened conditionality and underfunded transfer size.
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S8. Kenya — CT-OVC

Field	Extraction
ID	S8
Type	Feasibility study (mCCT for immunisation) + Transfer Project impact evidence
Citation	Brunet-Jailly, L. et al. (2016). "The feasibility of using mobile-phone based SMS reminders and conditional cash transfers to improve timely immunisation in rural Kenya." Plus: Handa, S. et al. (2014). <i>PLOS ONE</i> .
Sections served	7, 9
Country	Kenya
Programme name	CT-OVC (Cash Transfer for Orphans and Vulnerable Children)
Year adopted → status	2004 pilot / 2007 scaled → Active
Form of CCT	Officially unconditional but with soft messaging
Introduced by / rationale	Government of Kenya + UNICEF + DFID. Rationale: HIV/AIDS orphan crisis response.
Programme scale	~350,000 households (2020)
Transfer size in USD	USD 20/household/month (KES 2,000–4,000)
What was achieved	(1) Reduced odds of sexual debut by 31% among young people 15–25; 42% for females vs 26% for males. (2) No effect on condom use, number of partners, transactional sex. (3) Mobile-phone mCCT feasible for improving immunisation timeliness. (4) Increased consumption, food expenditure, school enrollment. (5) Reduced depressive symptoms by 24% among adolescents. (6) Impacts indirect (cash to caregiver, no conditions) — may work through keeping young people in school.
Where it fell short / limits	(1) Small scale relative to need. (2) No conditionality enforcement. (3) Transfer size modest (~20% of consumption). (4) HIV/AIDS context specific.
One-line what it adds	Africa benchmark — tests whether unconditional cash produces similar effects to conditional, with HIV/AIDS outcome evidence.

S9. Malawi — Zomba Cash Transfer Programme

Field	Extraction
ID	S9
Type	RCT comparing conditional, unconditional, and control — the first such experiment anywhere
Citation	Baird, S., McIntosh, C. & Özler, B. (2011). "Cash or Condition? Evidence from a Cash Transfer Experiment." <i>Quarterly Journal of Economics</i> 126(4): 1709–1753. Working paper: WPS5259, World Bank (2010).
Sections served	7, 9
Country	Malawi
Programme name	Zomba Cash Transfer Programme (research experiment)
Year adopted → status	2007–2009 (research experiment completed). Not a government programme.
Form of CCT	RCT with 3 arms: (1) CCT conditional on school attendance, (2) UCT unconditional, (3) Control. Targeted at adolescent girls.
Introduced by / rationale	World Bank research team + Malawian government cooperation. R' rationale: the first-ever rigorous test of the marginal contribution of conditionality.
Programme scale	176 enumeration areas randomly assigned: 88 treatment (split CCT/UCT) + 88 control.
Transfer size in USD	~USD 5–15/household/month
What was achieved	THE definitive test of conditionality: (1) CCT outperforms UCT on schooling: UCT effect was only 43% as large as CCT. CCT also improved learning outcomes; UCT showed no detectable learning gains. (2) UCT outperforms CCT on teenage pregnancy/marriage: Pregnancy reduced 34%, marriage reduced 48% in UCT arm. CCT had no impact. (3) The tradeoff: CCT improves human capital for "compliers" but denies transfers to "at-risk" girls who drop out. (4) Design implication: A CCT that transitions to UCT for older teenagers would minimise the tradeoff.
Where it fell short / limits	(1) Research programme, never scaled. (2) Targeted only at adolescent girls. (3) Short-term (2-year). (4) Single country — Malawi's low school quality may mean conditionality has moreD more "room to move" on enrollment but less on learning. (5) Self-reported enrollment diverged from school-confirmed.
One-line what it adds	The definitive test of whether conditionality delivers value over unconditional — answer: yes for schooling, no for teenage pregnancy/marriage, with a design implication to blend both.

S10. South Africa — HPTN 068 + Child Support Grant

Field	Extraction
ID	S10
Type	Peer-reviewed RCT evaluation
Citation	Kilburn, K., Ferrone, L., Pettifor, A., Wagner, R., Gómez-Olivé, F.X. & Kahn, K. (2020). "The Impact of a Conditional Cash Transfer on Multidimensional Deprivation of Young Women: Evidence from South Africa's HPTN 068." <i>Social Indicators Research</i> 151: 865–895.
Sections served	7, 9
Country	South Africa
Programme name	HPTN 068 (research trial) + Child Support Grant (mainline unconditional)
Year adopted → status	HPTN 068: 2011–2015 trial / CSG: 1998 → Active
Form of CCT	HPTN 068 conditional (school enrollment 80%) / CSG unconditional
Introduced by / rationale	HPTN 068: NIH + academic team. CSG: Government of South Africa post-apartheid.
Programme scale	CSG: 13 million children (2023) — one of world's largest UCTs. HPTN 068: RCT in rural Mpumalanga.
Transfer size in USD	CSG: ~USD 25/child/month. HPTN 068: ~USD 15–20/month conditional on school attendance.
What was achieved	(1) HPTN 068 reduced multidimensional deprivation among young women — even on dimensions not directly targeted. (2) Breaks link between school attendance and child labour. (3) CSG as contrast: world's largest unconditional child grant alongside research CCT — unique policy laboratory.
Where it fell short / limits	(1) HPTN 068 is a research trial, not scaled. (2) South Africa's unique institutional capacity (SASSA) enables scale others cannot replicate. (3) No significant effect on HIV incidence despite being primary trial endpoint.
One-line what it adds	Contrasts research CCT with the world's largest unconditional child grant — African evidence on whether conditionality adds value.

S11. Tanzania — PSSN

Field	Extraction
ID	S11
Type	RCT (HIV/STI prevention)
Citation	de Walque, D. et al. (2012). "Incentivising safe sex: a randomised trial of conditional cash transfers for HIV and sexually transmitted infection prevention in rural Tanzania."

Sections served	7, 9
Country	Tanzania
Programme name	PSSN (Productive Social Safety Net)
Year adopted → status	2013 → Active
Form of CCT	Hybrid: public works + cash transfer with soft conditions
Introduced by / rationale	Government of Tanzania (TASAF) + World Bank. Rationale: broader poverty reduction, combines cash with productive inclusion.
Programme scale	1.1 million households (2020)
Transfer size in USD	USD 10–25/household/month depending on components
What was achieved	(1) Africa's leading hybrid model — cash + public works. (2) CCT for HIV prevention showed short-term behaviour change but effects did not persist after payments stopped. (3) PSSN tests whether combining cash with productive inclusion works better than cash alone.
Where it fell short / limits	(1) HIV prevention CCT effects transient — behaviour reverts when payments end. (2) Administrative complexity of hybrid design. (3) Public works quality variable. (4) Scale-up challenges in low-capacity context.
One-line what it adds	Africa's leading hybrid model — tests whether cash + public works works better than cash alone, and whether CCTs can change sexual behaviour (answer: temporarily).

N1. Nigeria — COPE

Field	Extraction
ID	N1
Type	Research article (peer-reviewed, BMC)
Citation	Okoli, U., Morris, L., Oshin, A., Pate, M.A., Aigbe, C. & Muhammad, A. (2014). "Conditional cash transfer schemes in Nigeria: potential gains for maternal and child health service uptake in a national pilot programme." <i>BMC Pregnancy and Childbirth</i> 14: 39.
Sections served	8, 9
Country	Nigeria* Nigeria
Programme name	COPE (In Care of the People) / SURE-P MNCH CCT pilot
Year adopted → status	2007 (COPE) → 2015 closed. SURE-P pilot 2012–2015 also closed.

Form of CCT	Classical conditional (school + health for COPE; maternal health for SURE-P)
Introduced by / rationale	Yar'Adua administration + UNICEF technical support (COPE). SURE-P: post-fuel-subsidy-reinvestment under Jonathan administration.
Programme scale	COPE: ~22,000 households across 12 states at peak (very small). SURE-P pilot: 37 PHCs in 9 states.
Transfer size in USD	COPE: USD 10–25/household/month (NGN 1,500–5,000). SURE-P: up to NGN 5,000 (~USD 30) per woman for completing ANC + skilled delivery + PNC.
What was achieved	(1) SURE-P CCT pilot: Geographic targeting avoids costly means-testing. (2) Transfer sized to cover out-of-pocket costs of facility delivery. (3) Demonstrated feasibility of CCT for MNCH in Nigerian context.
Where it fell short / limits	(1) Never scaled — COPE closed 2015 without formal evaluation. (2) Targeting problems documented. (3) Supply-side constraints — PHC quality poor. (4) No lasting institutional legacy — lessons not incorporated into NASSP-HUP design.
One-line what it adds	Nigeria's first CCT attempt — the failed pilot that preceded NASSP, and the lesson that pilot evidence does not guarantee scale-up.

N2. Nigeria — Investigative Journalism on NASSP

Field	Extraction
ID	N2
Type	Investigative journalism (ICIR)
Citation	ICIR (2018). "INVESTIGATION: Poorest Nigerians deprived of federal government's cash transfer." July 5, 2018. https://www.icirnigeria.org/investigation-poorest-nigerians-deprived-of-federal-governments-cash-transfer/
Sections served	8, 9
Country	Nigeria
Programme name	NASSP-HUP
Year adopted → status	2016 → Active (with suspensions)
What was achieved (ground-truth)	(1) Payment delays and irregularities: Banks contracted as PSPs had discrepancies; contract terminated, shifted to manual cash at LGAs. Miss the day, forfeit the payment. (2) Inconsistent payment periods: Most beneficiaries received only 6 months while some received 10 months. (3) Exclusion errors documented. (4) No grievance mechanism accessible at time of investigation.

Additional sources	WB ISR (June 2022): Confirms delays, notes transition to digitised NUBAN accounts. NASSCO statement (2025): Digital payment transition tied to NIN verification slowed disbursements but necessary to "eliminate ghost beneficiaries."
Where it fell short / limits	(1) 2018 investigation predates 2020 restructuring. (2) Covers only Niger and Oyo states. (3) Does not assess programme impact on poverty.
One-line what it adds	Ground-truth counterweight to the World Bank institutional narrative — documents delivery failures that official reports acknowledge only in abstract terms.

N3. Nigeria — N-Power Cross-Reference (from Brief 4)

Field	Extraction
ID	N3
Type	Cross-reference to Brief 4 (Independent M&E Review)
Citation	Mashood, U.A. (2024). "N-Power: An Independent Monitoring and Evaluation Review." Brief 4, social-protection-me-nigeria.
Sections served	8
Country	Nigeria
Programme name	N-Power (graduate employment programme, sibling NSIP programme)
Year adopted → status	2016 launched → January 2024 suspended (12.5 months) → 2025/26 relaunch under "Renewed Hope"
Form of CCT	Unconditional stipend (NGN 30,000/month, no conditions attached)
Programme scale	Batch A: 200,000 (2016). Batch B: 300,000 (2017/18). Batch C Stream 1: 510,000 (2020). Batch C Stream 2: 490,000 (2020). Cumulative claim: 1.5M beneficiaries. Verified: only Batches A and B broadly corroborated; Batch C deployment unverified.
Stipend erosion	Nominal NGN 30,000 fixed since 2016. Real value (food-CPI deflated): 2016: NGN 30,000 → 2019: NGN 19,380 → 2022: NGN 11,441 → 2023: NGN 8,940 → 2024: NGN 6,663 → 2025: NGN 5,457 → 2026: NGN 3,000–5,000. Erosion of 82–90% by 2026. Restoration would require NGN 180,000–300,000/month.
Ghost beneficiaries	9–18% contamination band (up to ~90,000 payroll ghost names documented by independent reporting).
Post-programme unemployment	41.4% of sample still unemployed, only 7.1% in wage employment (Osimen et al. 2025, n=99 Lagos beneficiaries). Caveat: small, single-city sample.
January 2024 suspension	Promised 6 weeks; lasted ~12.5 months. Over NGN 30bn arrears. NGN 108bn arrears to 400,000 beneficiaries reported (Jul 2024). No published panel findings after suspension.

Key finding	The stipend erosion is the least contestable finding. The N-Power stipend had no automatic CPI-adjustment, unlike the companion World Bank-financed NASSP design which contained an automatic adjustment principle.
One-line what it adds	Sibling NSIP programme showing what happens when a cash transfer has no indexation mechanism — the same risk NASSP-HUP faces with its NGN 5,000 flat transfer.

X1. Cross-cutting — Conditional vs Unconditional

Field	Extraction
ID	X1
Type	Systematic review (peer-reviewed)
Citation	Bastagli, F. et al. (2016, ODI review Ch 7 + #21): "Conditional, Unconditional and Everything in Between: A Systematic Review of the Effects of Cash Transfer Programs on Schooling Outcomes."
Sections served	9, 10
What was achieved	(1) Both CCTs and UCTs improve school enrollment and attendance vs no transfer. (2) Effect sizes always larger for CCTs than UCTs, but the difference is not always statistically significant. (3) By enforcement strength: Explicitly conditional programmes with monitored compliance have ~60% improvement in odds of enrollment vs no/some conditions. (4) Test scores: Effectiveness small at best — conditionality doesn't fix learning quality. (5) Messaging matters as much as enforcement.
Where it fell short / limits	(1) Cannot disentangle which aspect of conditionality drives results. (2) Longer-term outcomes under-studied. (3) Context dependency.
One-line what it adds	Does conditionality earn its administrative cost? Yes for enrollment, especially with enforcement; no for learning; messaging may matter as much as monitoring.

X2. Sub-Saharan Africa Regional

Field	Extraction
5	ID
Type	Systematic review + scoping review
Citation	(1) Systematic review on CCT child health SSA (#127). (2) Ngamasana & Moxie (2024). "Cash transfer, maternal and child health outcomes: a scoping review in sub-Saharan Africa." <i>Global Health Action</i> 17(1).
Sections served	7, 9, 10

What was achieved	(1) CTs in SSA increase health service utilisation (ANC, immunisation, facility delivery). (2) Nut< nutritional outcomes improve (dietary diversity, food security) but anthropometric outcomes (stunting, wasting) show limited significant effects. (3) Conditionality enforcement weak or absent in most SSA programmes — African CTs tend toward unconditional or soft-conditional. (4) Supply-side constraints are the binding limitation. (5) UCT outcomes in SSA comparable to CCT where supply-side weak.
Where it fell short / limits	(1) Evidence base thinner than Latin America. (2) Heterogeneity across SSA. (3) Long-term evidence absent. (4) MCH outcomes beyond nutrition inconclusive.
One-line what it adds	Africa-specific aggregate — the evidence that in SSA's weak-supply context, the CCT instrument mutates toward unconditional, and conditionality adds less value than in Latin America.

Supporting: #135 — Ghostwriting of Global Policy Script

Field	Extraction
ID	#135 (Supporting for Section 1)
Type	Peer-reviewed interpretive analysis
Citation	Heimo, L. and Syväterä, J. (2022). "The ghostwriting of a global policy script: international organizations and the discursive construction of conditional cash transfers." Critical Policy Studies 16(1): 79-96. DOI: 10.1080/19460171.2021.1967178
Sections served	1
Key argument	The World Bank, IADB, and IFPRI discursively constructed the CCT narrative in their policy documents. The "Mexico and Brazil created CCT" origin story conceals IOs' involvement in designing and financing pre-Progresas programmes. The WB resorted to "ghost-writing" the CCT script, expurgating its own central role, as a result of its legitimation deficit after the unpopular structural adjustment era.
Evidence	(1) WB and IDB designed and financed programmes in Honduras and Nicaragua before Progresas. (2) IFPRI produced key research that informed Progresas's design. (3) Early IO-produced studies crafted the narrative that attributed CCT origination solely to Mexico and Brazil. (4) Leisering (2019) acknowledges WB's central role but pays little attention to IO involvement prior to Progresas.
One-line what it adds	Directly challenges the "CCT was not an IO invention" argument. Essential counterpoint for Section 1 debate. Heimo and Syväterä vs Fenwick + Hanlon.

Supporting: #53 — Cash Transfers and Child Schooling (Burkina Faso RCT)

Field	Extraction
ID	#53 (Supporting for Sections 7, 10)
Type	World Bank Policy Research Working Paper (RCT)
Citation	Akresh, R., de Walque, D. & Kazianga, H. (2013). "Cash Transfers and Child Schooling: Evidence from a Randomized Evaluation of the Role of Conditionality." WPS6340.
Sections served	7, 10
Country	Burkina Faso
Programme name	Nahouri Cash Transfers Pilot Project (NCTPP)
Design	4-arm RCT: CCT to fathers, CCT to mothers, UCT to fathers, UCT to mothers. Two-year pilot.
Key findings	(1) CCT more effective for "marginal children" : girls, younger children, lower ability children — the ones initially less likely to be enrolled. (2) UCT equally effective as CCT for "non-marginal children" : boys, older children, higher ability children — the ones parents already prefer to invest in. (3) Conditionality binding where enrollment is low (Burkina F4 Faso); inframarginal where enrollment is high (Morocco — Benhassine et al. 2015 found no CCT-UCT difference). (4) Recipient gender matters but interacts with conditionality.
Contrast with Baird et al. Malawi	Baird: CCT > UCT for schooling, but UCT > CCT for teenage pregnancy. Akresh: CCT > UCT specifically for marginal children, UCT = CCT for non-marginal. Together: conditionality's value depends on who the target population is and what the binding constraint is.
One-line what it adds	Second rigorous conditionality RCT in Africa — refines the Baird finding by showing conditionality specifically benefits marginal children, while UCT matches CCT for children already likely to attend.

Supporting: #129 — Did CCTs Go Too Far?

Field	Extraction
ID	#129 (Supporting for Section 5, 10)
Type	Policy research paper

Citation	Stampini, M. & Tornarolli, L. (2012). "The Growth of Conditional Cash Transfers in Latin America and the Caribbean: Did They Go Too Far?"
Sections served	5, 10
One-line what it adds	The question of over-expansion: whether CCTs in Latin America expanded beyond their fiscal and institutional capacity. Critical for Section 10 verdict.

Supporting: #31 — Long-term Impacts of CCTs in Latin America

Field	Extraction
ID	#31 (Supporting for Section 9)
Type	Review article
Citation	"Long-term impacts of conditional cash transfers in Latin America: Review of the evidence."
Sections served	9
One-line what it adds	Essential for Section 9 — long-term evidence beyond the short-term impact studies that dominate the literature.

Supporting: #150 — Oportunidades Birthweight Impact

Field	Extraction
ID	#150 (Supporting for Section 3)
Type	Peer-reviewed (Tropical Medicine & International Health)
Citation	Barber, S.L. & Gertler, P.J. (2008). "The impact of Mexico's conditional cash transfer programme, Oportunidades, on birthweight." <i>Tropical Medicine & International Health</i> .
Sections served	3
One-line what it adds	+127.3g birthweight, 4.6pp reduction in low birthweight — the specific health impact evidence for Progres/Oportunidades.

GAP FLAGS AND UNCERTAINTIES

1. **F3 Levy 2006 book — SKIPPED.** Paywalled. F1 + S1 cover Progres/Oportunidades adequately. If you later want the architect's own voice, you would need to source this.
2. **N3 N-Power unemployment rate (41.4%)** — Single cross-sectional survey of 99 Lagos beneficiaries (Osimen et al. 2025). Not nationally representative. Included because Brief 4 uses it, but should be footnoted as limited sample.

3. **Ghana LEAP endline quantitative effect sizes** — Qualitative findings obtained from 2020 endline summary (raised consumption, decreased poverty, reduced IPV, no stunting impact). Precise coefficient estimates not extracted; full PDF would need additional hunting if you want them.
4. **NASSP-HUP transfer amount constancy** — F4b PAD states NGN 5,000/month. F4a ICR does not report a change between 2016–2024. I cannot confirm from primary data whether the amount ever changed.
5. **Nigeria 2024 suspension and Renewed Hope reset details** — The January 2024 NSIP suspension is documented from Brief 4 (N-Power) and ICIR/WB ISR (NASSP). The Renewed Hope re-framing is documented in F4b PAD (approved 2022, operationalised under new administration). Specific political details of the Tinubu transition's effect on NASSP are not in any source currently in the matrix.

TOTAL: 24 source tables (F1, F2, F4a, F4b, F5, F6, S1–S11, N1–N3, X1, X2) + 5 supporting papers (#135, #53, #129, #31, #150) + 1 gap (F3 skipped) = 30 entries.